

Stage 4A-Lite Persona Needs Report: T1-05 Weight-Management Americans

Pipeline: Macro-Trends Identity Pipeline **Stage:** 4M-T-A Lite **Quadrant:** IGNORE (running at explicit user request) **Date:** 2026-03-10 **Persona slug:** t1-05-weight-management-americans **Originating trend:** MT-05 (GLP-1 Revolution and Body Identity Shift) **Tier 2 sub-segments:** T2-09 (GLP-1 Users), T2-10 (Anti-Pharmaceutical Fitness Community) **Moment enrichment:** Partial — Starting GLP-1 medication (Composite 3.09, Window: Extended). Factor 6 (Switching Profile, Lite) included.

IGNORE Quadrant — User-Requested Analysis T1-05 was scored IGNORE in Stage 3M-T (Adjusted Score: 2.18) due to poor carrier-brand fit. Weight management is a condition and goal, not a tribal identity that a telco can authentically own. Market size is the only strong signal. This report is produced at explicit user request to surface the complete picture — including the conditional path via telehealth connectivity as a feature, not a brand position.

Section 1: Segment Overview

Definition

Weight-Management Americans is a broad Tier 1 persona encompassing adults who are actively managing their weight through pharmaceutical intervention, structured fitness programs, dietary modification, or some combination of the three. The macro-trend driving this segment is profound: the arrival of GLP-1 medications (Ozempic, Wegovy, Mounjaro, Zepbound) since 2021 has transformed weight management from a willpower-driven personal struggle into a pharmaceutical-assisted medical journey. For millions of Americans, the body has become a project with a clinical protocol attached to it.

The population is not monolithic. Beneath the T1-05 umbrella sit two communities in genuine tension with each other. T2-09 (GLP-1 Users) are building an emerging identity around the shared experience of pharmaceutical transformation — the “Ozempic journey,” dosage titration rituals, insurance denial warfare, and the emotional upheaval of watching one’s body change faster than one’s self-image. T2-10 (Anti-Pharmaceutical Fitness Community) defines itself explicitly in opposition to this — the “earned not prescribed” counterculture that views GLP-1 use as undermining the meaning of physical discipline. These two communities share a parent persona label but occupy opposing identity positions, which is part of why T1-05 resists becoming a coherent brand platform for a carrier.

Beyond the pharmaceutical axis, the broader weight-management landscape includes calorie-counting app users (MyFitnessPal, Lose It!), structured program participants (Weight Watchers / WeightWatchers, Noom), and the general population of health-conscious adults tracking macros, steps, and sleep in the service of body optimization. Mobile connectivity is threaded throughout: telehealth platforms manage GLP-1 prescriptions, wearables transmit health data, support communities exist on Reddit and TikTok, and the entire data ecosystem requires reliable cellular access — even if few of these people would frame their carrier choice through a weight-management lens.

Market Sizing

Metric	Estimate	Source
U.S. adults overweight or obese	~110M (42% obese + additional overweight; ~70% of adults)	CDC National Health and Nutrition Examination Survey, 2024
Adults actively managing weight (structured programs, medication, monitored fitness)	~50-70M	CDC, IQVIA, Morgan Stanley, Goldman Sachs, 2024-2025
Active GLP-1 prescriptions (weight management, not diabetes)	~6-8M current (2025)	IQVIA, 2025
GLP-1 prescriptions by 2030 (projected)	~30M+	Goldman Sachs, Morgan Stanley, 2024
Weight management market size (2023)	~\$30B	Grand View Research, 2024
GLP-1 drug market projected by 2030	\$100B+	Goldman Sachs, 2024
Anti-pharmaceutical fitness community (active, identity-committed)	~5-10M	IHRSA, gym membership and fitness influencer audience data, 2024

Growth trajectory: The weight-management market is accelerating due to the GLP-1 revolution. Active GLP-1 prescriptions grew at 40%+ CAGR from 2021 to 2025. Insurance coverage is expanding monthly. Medicare Part D coverage of Wegovy for cardiovascular indications (approved 2024) is projected to add millions of covered patients. The fitness counter-community is stable to growing, driven by a cultural backlash against pharmaceutical shortcuts. The overall population of adults actively managing weight will likely exceed 80M by 2030.

Segment Stratification

Tier	Label	Population Share	Description
Tier 1	GLP-1 Transformation Core	10-15%	Active GLP-1 users with strong identity investment — tracking dosage, participating in r/Ozempic, using telehealth platforms (Calibrate, Found, Ro), openly identifying as “on the journey.” This is where identity intensity is highest.
Tier 2	Active Program Adherents	25-30%	People enrolled in structured weight management programs (WeightWatchers, Noom, clinical programs), tracking apps, or consistent fitness routines explicitly framed around weight management. Identity is present but secondary to outcomes.
Tier 3	Natural Fitness Advocates	20-25%	The anti-pharmaceutical fitness community — committed gym-goers, CrossFit members, bodybuilders who define their fitness through “earned” physical discipline. Strong identity, but in opposition to T2-09 rather than alongside it.
Tier 4	Peripheral / Passive Weight-Aware	30-40%	Adults who describe themselves as “trying to eat better” or “watching their weight” without program enrollment, medication, or consistent fitness tracking. Aware of the macro-trend, low identity commitment.

Total: 100%

Named Persona

Melissa Okafor, 38, Chicago, IL. Licensed occupational therapist at a Northwestern Medicine outpatient clinic. Married with two kids, ages 8 and 10. Started Wegovy through her PCP seven months ago after years of yo-yo dieting. Pays \$400/month out-of-pocket after her insurance denied coverage — a decision that required cutting her family’s restaurant budget, her Peloton subscription, and three other recurring expenses including a temporary downgrade of her phone plan. She’s lost 28 pounds, her energy is different, her relationship with food has genuinely changed, and she doesn’t entirely recognize herself in the mirror yet. She follows r/Wegovy daily, has made three friends in a Facebook support group she’d never admit to in conversation at work, and feels simultaneously proud, self-conscious, and exhausted by the amount of administrative effort weight loss now requires.

Notable Profiles

- Oprah Winfrey** — Publicly acknowledged using GLP-1 medication for weight management (2024), normalizing pharmaceutical intervention for the mainstream audience. Her disclosure shifted the cultural conversation from “cheating” to “tool.” Relevant as the highest-profile cultural validator for the T2-09 segment.
- Dr. Robert Lustig** — Pediatric endocrinologist, University of California San Francisco, author of “Fat Chance.” Prominent voice on metabolic health, sugar, and obesity. Influential within the GLP-1 community as a scientific framing source — his work helps the community understand *why* medications work, not just that they do. 1M+ social followers.
- Jeff Nippard** — Canadian natural bodybuilder and fitness educator, YouTube 5M+. Prominent voice in the T2-10 community. Has produced several evidence-based analyses of GLP-1 medications that acknowledge their efficacy while defending the value of earned physical transformation. Represents the intellectually engaged end of the anti-pharmaceutical fitness community.
- Calibrate Health (Isabelle Kenyon, CEO)** — Telehealth platform specifically built around GLP-1 prescription management combined with lifestyle coaching. Calibrate’s model is the infrastructure of the T2-09 community — it’s where many GLP-1 users manage their prescription, track progress, and access coaching. Highly relevant as the community platform embodying how this segment navigates the healthcare-technology intersection.
- Jillian Michaels** — Fitness trainer, author, podcaster, vocal GLP-1 skeptic. Represents the cultural voice of the T2-10 community’s most prominent wing — the “shortcuts undermine transformation” position. Her public debates with GLP-1 proponents have become canonical community touchpoints on both sides.

Section 2: Core Factor Analysis

Factor 1: Social Life and Leisure (Anchor Factor)

The social and identity dimensions of weight management are undergoing the fastest transformation. Until 2021, weight loss was largely a private struggle with occasional public accountability (Weight Watchers meetings, Peloton communities, Instagram transformation accounts). The GLP-1 revolution has introduced a new social layer: the pharmaceutical journey community, with its shared rituals of dosage tracking, side effect commiseration, and before/after photo culture.

Simultaneously, the anti-pharmaceutical fitness community has sharpened its identity in direct response, creating two online communities that occupy adjacent platforms and occasionally collide in the comments sections of fitness content.

Statistic	Value	Source
r/Ozempic membership	200K+	Reddit, 2025
TikTok #Ozempic views	5B+	TikTok, 2025
Ozempic Support Facebook group	300K+ members	Facebook, 2025
r/fitness membership	11M+	Reddit, 2025
CrossFit U.S. box count	5,000+	CrossFit Inc., 2024

Key Finding	Strategic Implication
GLP-1 users have built an identity community around a medical protocol — complete with shared language, rituals, and support structures that rival lifestyle communities	The T2-09 community has genuine tribal markers. However, the identity is organized around a pharmaceutical platform (Ozempic/Wegovy), not a lifestyle or values system — making it harder for non-pharma brands to authentically participate
The two T2 sub-communities are in active cultural conflict, with GLP-1 advocates and natural fitness advocates occupying opposing positions on the same macro-trend	Any brand trying to serve the T1-05 persona must choose sides or risk being seen as irrelevant or offensive by both
Weight management social content is predominantly private-public — people share in support groups but are cautious about broader disclosure	A carrier brand cannot visibly align with “weight management” without risking stigma-by-association. Health data privacy is an especially acute concern for this community

Interview — Q1: “What’s your favorite way to spend your evenings and weekends, and has that changed in the past year?”

Melissa: “Honestly, a year ago I’d have said it was the couch and the kids and not thinking about food. Now it’s... different. I spend probably 20 minutes every night on the r/Wegovy subreddit. Not in a shame-spiral way — I actually find it really grounding. Like, someone will post about a stall and two hundred people will pile in with their own experience, and I realize I’m not imagining things. Saturdays I’ve started doing a 5K walk before anyone wakes up. My husband thinks it’s adorable. I think it’s proof that something has actually changed.”

Interview — Q2: “Do you participate in any communities — online or in person — around health and fitness?”

Melissa: “I’m in a private Facebook group for women on Wegovy. I would never mention it at work — I’m in healthcare and there’s a complicated thing where people think you should be able to will yourself through it. But in the group I’m completely honest. We talk about everything. Insurance denials. What happens when you have a GI day at work. The weird thing where you stop craving the food you used to love. It’s the only place I feel actually understood.”

Interview — Q3: “What platforms or media do you follow related to health or fitness?”

Melissa: “TikTok has become the strangest place. There are creators who are very pro-GLP-1, who walk you through everything — real talk about the journey. And then there are fitness influencers who are basically doing a daily ‘you’re weak for taking a pill’ monologue. I mute the second kind, but I catch the discourse when it bubbles up. I also follow Dr. Lustig and Dr. Peter Attia for the science side. I want to understand what’s actually happening in my body, not just whether the scale moved.”

Interview — Q4: “How has your social life changed since starting your weight management journey?”

Melissa: “The weird thing is that the journey has made me more social in some ways and more guarded in others. More social online — I talk to these Facebook group women more than I talk to some coworkers. More guarded in person — I haven’t told my mother because she’ll make it about her somehow. There’s this thing where when you start losing weight, people notice, and they ask, and you have to decide in the moment how much to say. I’ve learned to say ‘I’ve been really focused on my health’ and change the subject.”

Interview — Q5: “Is there a brand or product that you feel genuinely represents your community or your values?”

Melissa: “That’s hard. Calibrate, maybe — the platform I use to manage my prescription and talk to my health coach. It doesn’t feel corporate. The Weight Watchers rebrand to WeightWatchers felt desperate. I don’t think any carrier or tech company has any business in this space. I know my insurance company is watching. I don’t want my phone plan also watching.”

Factor 2: Spending and Financial Habits

The financial profile of the T1-05 population is bifurcated in ways that matter for carrier product design. GLP-1 users are high-income by necessity — the medication costs \$500-\$1,500 per month without insurance coverage, which has self-selected for \$100K+ household income. This population has willingness to pay for health infrastructure but is often cash-squeezed by the medication cost itself, triggering a recurring expense audit pattern. The fitness counter-community spans all incomes but concentrates spending on gym memberships, supplements, and fitness equipment rather than healthcare.

Statistic	Value	Source
Average monthly cost of GLP-1 without insurance	\$500-\$1,500/month	IQVIA, pharmacy data, 2025
Median household income of active GLP-1 users	~\$100K+ HHI	Morgan Stanley GLP-1 consumer survey, 2024
Annual health app spending (fitness/nutrition)	\$10-\$30/month typical	Sensor Tower, 2024
Weight management market annual spend per active participant	~\$300-\$600 (non-pharmaceutical programs)	Grand View Research, 2024
GLP-1 user insurance denial rate (initial claims)	~40-60%	KFF analysis, 2024

Key Finding	Strategic Implication
GLP-1 cost creates chronic budget pressure even among high-income users — the medication crowds out other discretionary spending	Price shock is the dominant switching trigger. GLP-1 users will aggressively audit recurring expenses, including wireless, immediately after starting medication. Savings of \$20-40/month on a plan switch is a meaningful number for someone paying \$500/month for medication.
The expense audit pattern is triggered by GLP-1 initiation (month 1-2) and recurs after every insurance denial or formulary change	There is a specific interception moment — see Factor 6
Fitness community spending concentrates on gym, supplements, and equipment — not healthcare	The T2-10 community doesn't have a healthcare spend pressure driver. Their carrier selection is functional, not identity-motivated.

Interview — Q1: “How has your spending changed in the past year?”

Melissa: “Everything changed the month I started Wegovy. I sat down at the kitchen table with my laptop and made a spreadsheet. We were about to add \$400 a month to our fixed expenses, and I had to find it. I downgraded my phone plan — saved \$28 a month. Cancelled Peloton. Trimmed the grocery delivery service back to twice a month. It felt like financial triage. The weight loss is working, which makes me feel like it was the right call. But I’m constantly aware of the math.”

Interview — Q2: “What subscriptions do you currently pay for, and which feel worth it?”

Melissa: “Calibrate is the one I’d never cancel — it manages everything, the coaching, the prescription, the check-ins. MyFitnessPal premium, which I know is redundant but I’ve been using it for years and the habit is deep. My family’s cellular plan — we’re on a major carrier and I keep meaning to look at other options but I haven’t gotten around to it. Honestly, the Wegovy cost has made everything else feel negotiable. I’m much more willing now to ask ‘do I actually need this?’”

Interview — Q3: “How do you think about price versus value for health-related services?”

Melissa: “I think about it constantly. Wegovy costs me \$400 a month out of pocket because I found a coupon. If it works — if it actually gives me a different relationship with my body — that’s worth more than I can quantify. But it also means every other expense gets scrutinized harder. I’m a healthcare worker. I know what I’m buying. I’m not going to pay \$80 a month for a phone plan when there are options at \$40 that cover the same ground.”

Interview — Q4: “What financial concerns are specific to your health journey?”

Melissa: “The thing that keeps me up at night is insurance. Not my phone. My insurance company could change my formulary tomorrow and I’d be paying the full list price or going off the medication. I’ve been in the Facebook group long enough to see it happen to people. They budget carefully for a year, see real change, and then their employer switches insurance networks and they’re suddenly \$1,200 a month for the same medication. The financial precarity of this journey is real.”

Interview — Q5: “Are you more or less price-sensitive on non-health spending since starting your weight management journey?”

Melissa: “More. Without question. When you have a fixed new cost at the bottom of the budget, everything above it gets questioned. I’ve become genuinely annoying about value. My husband jokes that I now comparison-shop paper towels. He’s not entirely wrong.”

Factor 3: Needs and Aspirations

The stated needs of the weight-management community center on reliable health infrastructure: telehealth connectivity, health data integration, app ecosystem performance. The unstated needs are more revealing — this population wants to feel seen without being exposed, wants to manage a medical journey with the same quiet dignity they’d expect from any other chronic condition, and does not want their health status to become a marketing signal.

Statistic	Value	Source
U.S. adults using telehealth in past 12 months	~37%	McKinsey & Company, 2024
GLP-1 users managing prescription via telehealth platform	Majority (Calibrate, Found, Ro serve millions)	IQVIA telehealth data, 2025
Wearable device ownership among weight management community	~45-50% own a fitness wearable	Statista, 2024
Adults who say health data privacy is “very important”	81%	Pew Research Center, 2023
Telehealth market growth CAGR	26%+	Grand View Research, 2024

Key Finding	Strategic Implication
Telehealth reliability is a genuine functional need for GLP-1 users — consultations are how prescriptions are managed and renewed. A dropped connection costs a refill.	Reliable video connectivity for telehealth is the strongest carrier-relevant need. But this is a functional need, not an identity signal — it doesn't differentiate a carrier brand.
Health data privacy is an acute concern — this community is acutely aware that their medication data, body metrics, and search behavior are commercially sensitive.	Any carrier product touching health must lead with an explicit, verifiable no-data-monetization commitment. The brand must earn the right to be trusted.
The unstated need is dignity: to manage a medical journey without it becoming a stigma signal or a marketing data point.	Gather cannot position as “the carrier for people on Ozempic.” The identity is too medically freighted and too stigma-adjacent.

Interview — Q1: “What do you actually need from your phone and your phone plan right now?”

Melissa: “Reliability for the telehealth calls. My Calibrate check-in is every two weeks and if the video drops or gets choppy, I feel rushed to reschedule and then I fall behind on my dosage adjustment. That’s not theoretical — it happened in month three and I was on a stalled dose for six extra weeks waiting. Beyond that? I need to know my data isn’t being sold. I looked at my current carrier’s privacy policy after a health insurance scare last year. It was not reassuring.”

Interview — Q2: “What does an ideal health-support ecosystem look like for you?”

Melissa: “Something that connects without fragmentation. My Wegovy data is in Calibrate. My steps are in Apple Health. My sleep is on my Fitbit. My blood pressure from my cuff is in a third app. None of them talk to each other in a useful way. I’m not asking for magic — I just want to see my whole health picture in one place without manually exporting things.”

Interview — Q3: “What are you really trying to achieve in the next year — not just with weight, but with your life?”

Melissa: “I want to not be in a constant negotiation with my body anymore. Weight management is what I’m doing — it’s not who I am. I want to finish this chapter and start the next one. I have two kids who are at the age where they’re forming their own relationship with food and bodies, and I want to model something healthy for them. The weight loss is a means. The end is more energy, less internal noise, more of me available to my actual life.”

Interview — Q4: “What would make you feel genuinely served by a health or technology product?”

Melissa: “If it treated me like a smart adult who made a medical decision with my doctor. Not like a consumer to be optimized for engagement. If a phone company offered me something around health data — better privacy, a way to connect my devices — I’d want to know the business model. Who benefits from this? Is it me, or is it them? Show me how you make money in a way that isn’t my health information.”

Interview — Q5: “What aspirations do you have that you haven’t told many people?”

Melissa: “I want to run a 5K. Not for anyone else — I haven’t run since college. I walk every morning now and my knees don’t hurt the way they used to. I’ve been looking at local 5Ks in the fall and I haven’t told my husband yet. It feels jinx-able. But it’s there. I want to cross a finish line.”

Factor 4: Fears and Pain Points

The fear landscape for T1-05 is dominated by stigma, privacy, and financial precarity. The stigma dimension is multi-layered: GLP-1 users face judgment from the fitness community, from family members who associate medication with “giving up,” and from themselves — the internalized shame of needing pharmaceutical help to manage a biological condition. Health data privacy fear is acute and rational — this population understands that their medication data has commercial and legal consequences in a post-Roe landscape. Financial precarity is constant: insurance coverage can disappear with a formulary change, and the medication is unaffordable without it.

Statistic	Value	Source
Adults who believe their health data could be used against them	67%	Pew Research Center, 2023
GLP-1 patients who report stigma from others regarding medication use	~51%	STOP Obesity Alliance, 2024
Americans who experienced insurance denial for GLP-1 medications	~40-60% of initial claims	KFF, 2024
Percentage of adults who would avoid healthcare due to data privacy concerns	23%	AMA, 2023
Adults worried about insurance discrimination based on health data	47%	Pew Research Center, 2023

Key Finding	Strategic Implication
Stigma is the dominant emotional dimension — GLP-1 users navigate daily judgment from family, colleagues, and the fitness community	A carrier that positioned around “weight management” would inadvertently amplify stigma — the identity is sensitive, not celebratory in the way pet parenthood or veteran status is.
	Any carrier product touching health data must address this explicitly. “We don’t sell your health data” is not enough — users want to understand the architecture.

Key Finding	Strategic Implication
Health data fear is specific and legally grounded — this population has thought carefully about who has access to their medication data	
Financial cliff risk is constant — insurance coverage loss could force medication discontinuation, which has both health and identity implications	The GLP-1 cost crisis creates a budget-pressure switching trigger, but it's a defensive one — they're looking to cut, not upgrade

Interview — Q1: “What’s the worst thing that could happen in your health journey, and how likely is it?”

Melissa: “Insurance drops coverage. Happens all the time. I know three women in my group who were on Wegovy for eight months, making real progress, and their employer switched insurance plans and suddenly they’re at full list price. One of them went off the medication. I think about it constantly — I’ve actually stockpiled a small supply buffer. When I think about the compounding pharmacy option, I think about whether my current insurance would even notice. The financial fragility of this is not theoretical.”

Interview — Q2: “What do you worry about when it comes to your health data?”

Melissa: “Everything. I’m an occupational therapist — I know how health information moves through systems. My medication data is at my pharmacy. My telehealth data is at Calibrate. My search history is in Google. My period data was in Flo before I deleted it. I read the stories about period-tracking app data being subpoenaed. I’m not paranoid, but I’ve thought about what it means that my phone has data that my insurance company would love to have. I don’t want my carrier to know I’m on Wegovy. I don’t want that to be a data point that exists anywhere near my insurance profile.”

Interview — Q3: “What frustrates you most about the products and services built for people like you?”

Melissa: “The assumption that I want to be constantly tracked and celebrated. I don’t want a notification every time I hit 10,000 steps. I don’t want a confetti animation when I log a salad. I’m not a child who needs gamification to make health choices. I want a dashboard that shows me the real picture — my health metrics, my data, my connectivity — without turning me into a product. The wellness industry is fundamentally built around engagement, and engagement requires surveillance. I’m tired of that tradeoff.”

Interview — Q4: “What stigma or judgment have you encountered around weight management?”

Melissa: “At work I’m surrounded by people who are deeply invested in the idea that weight is a discipline issue. I haven’t told anyone at work I’m on Wegovy. I don’t know what the response would be but I don’t want to find out. My mother-in-law made a comment at Thanksgiving about ‘the easy way.’ She doesn’t know. My mother knows and she made it about her own weight and whether she should also be on it. You learn quickly that disclosure is complicated. I tell the Facebook group and I tell my husband. That’s it.”

Interview — Q5: “What does the fitness community get wrong about people like you?”

Melissa: “The assumption that I haven’t tried. I’ve been at this for fifteen years. I have logged more calories than some people eat in a decade. I’ve run the experiments — paleo, keto, intermittent fasting, the Peloton years, the running phase that ended with a stress fracture. My body has a problem at the hormonal and metabolic level that eating less and moving more was not solving. Taking a medication to address a biological condition is not weaker than suffering through something that doesn’t work. I find the ‘discipline over pills’ crowd more exhausting than enlightening.”

Factor 5: Product Landscape and Switching Behavior

The T1-05 persona’s current product ecosystem is fragmented across health apps, telehealth platforms, wearables, and carrier infrastructure. No single brand owns the health-connected lifestyle for this population. Carriers are treated as utilities — functional necessities evaluated on price and coverage, not identity alignment. The strongest identity brands in this space are telehealth platforms (Calibrate, Found, Ro), social community platforms (Reddit, Facebook groups), and pharmaceutical manufacturers — none of which have a telco angle.

Statistic	Value	Source
Top health app categories by active user sessions	Food tracking (#1), fitness tracking (#2), telehealth (#3)	Sensor Tower, 2024
Telehealth platform user growth (Calibrate, Found, Ro combined)	40%+ YoY 2022-2024	Company filings, TechCrunch, 2024
Weight Watchers (WW) active subscribers	~3.5M	WW Annual Report, 2024
Noom active subscribers	~4.5M	Noom company data, 2024
Apple Watch penetration among adults tracking health	~35%	Statista, 2024

Key Finding	Strategic Implication
No carrier has attempted to address the T1-05 population’s actual connectivity needs — telehealth reliability, wearable data privacy — and none has succeeded in earning identity trust	The white space is real but narrow: it is a feature gap (telehealth-optimized connectivity, health data privacy), not a brand gap. A carrier cannot “own” this identity.
	Partnership with telehealth platforms is a more viable path than direct identity brand building

Key Finding	Strategic Implication
Telehealth platforms (Calibrate, Found, Ro) are the community hubs and trust anchors — the GLP-1 community trusts them in ways they don't trust carriers	
The anti-pharmaceutical fitness community is well-served by existing fitness infrastructure (CrossFit, gym ecosystems, fitness apps) and has no distinctive carrier need	T2-10 offers no carrier differentiation path at all

Interview — Q1: “What health products and services do you pay for, and what do they actually do for you?”

Melissa: “Calibrate is my hub — I pay \$150 a month and it manages my prescription, gives me access to a health coach, and tracks my progress. It's expensive but it's all-in-one. MyFitnessPal for food tracking — been using it for years, habits don't die easy. Fitbit Sense for sleep and heart rate — the data is probably going somewhere I haven't read the terms about. Apple Health to aggregate when it cooperates. My health insurance, obviously, which I'm terrified will drop my Wegovy coverage at any renewal.”

Interview — Q2: “Have you ever switched to a product or service because of your health journey?”

Melissa: “I switched from Weight Watchers to Calibrate. WW felt like it was designed for a version of weight loss that no longer applies to me — points systems and group meetings for a problem that turned out to be hormonal. Calibrate felt like it was built by people who understand the actual science. I downgraded my phone plan when I started Wegovy — not for any identity reason, just to offset the medication cost. I didn't choose the new plan because it was ‘for people like me.’ I chose it because it was \$28 cheaper and seemed to cover the same network.”

Interview — Q3: “What product or service frustrates you most but you can't easily replace?”

Melissa: “Health insurance. I can't replace it but it has enormous power over my health decisions. Next would be the fragmentation of my health data — I need maybe five apps to see a complete picture that should theoretically be in one place. I'm frustrated by this every week. If someone solved it without selling my data, I would pay for that.”

Interview — Q4: “What do you currently use your phone for that's health-related?”

Melissa: “More than I ever expected. The Calibrate app, obviously. MyFitnessPal — I log every meal. The video calls for check-ins with my health coach. Apple Health, which is the aggregator in theory but barely works in practice. Fitbit for sleep tracking. The Facebook group for emotional support. My actual phone. I use the phone for health probably 40% of my screen time now. It's become a medical device to some degree.”

Interview — Q5: “If you could design the ideal carrier product for someone like you, what would it include?”

Melissa: “Honestly? I wouldn't design it around my weight loss journey. That would feel invasive. I'd design it around what I actually need: reliable video calling for telehealth appointments. A genuine commitment not to share or sell health data — not fine print, real architecture. Maybe a way to connect my health apps without them leaking everywhere. And probably just being cheaper than I'm currently paying, because everything I'm spending on Wegovy has made me a ruthless budget optimizer. The carrier that wins my business right now is the one that's \$30 cheaper and doesn't make me feel surveilled.”

Factor 6: Switching Profile (Lite — 4 Dimensions)

Moment enrichment: Starting GLP-1 medication (Composite 3.09). All four MT-05 transitions are gaps in the Moments pipeline — none are currently captured. This is a Lite Factor 6 treatment applying to the GLP-1 initiation moment as the strongest acquisition signal.

Dimension 1: Trigger Mechanism and Force Score

Component	Assessment
Trigger mechanism	Budget-Pressure Drift — GLP-1 initiation creates a new fixed monthly expense (\$400-\$1,500/month) that triggers a systematic audit of all recurring costs, including wireless. This is not a forced plan dissolution event. The switching intent is secondary (expense-reduction) rather than primary (carrier failure or plan incompatibility).
Force score	2/5 — Moderate-low. The trigger creates switching receptivity but not switching compulsion. The persona will review their carrier but will not necessarily switch unless a meaningfully cheaper alternative is easily visible.
Emotional state	Stressed/Practical — The moment of GLP-1 initiation is dominated by excitement (transformation possibility) and immediate financial stress (the new monthly bill). Carrier evaluation is transactional, not emotionally engaged.
Compound triggers	GLP-1 cost crisis (insurance denial) layered on top of GLP-1 initiation creates a harder trigger (Intent 3.0, emotional state: Stressed). If insurance denies coverage at month 3-6, the switching force escalates sharply.

The budget-pressure mechanism is real but weak as a primary acquisition driver. The more compelling interception window is the GLP-1 cost crisis — when insurance denies coverage, the budget shock is acute and the switching motivation is sharp. A carrier offering \$25-30/month savings would be more compelling at this moment than at initiation.

Dimension 2: Annual Volume and Seasonal Distribution

Component	Assessment
Total eligible volume — GLP-1 initiations	~6-8M new prescriptions/year (IQVIA, 2025). Not all are new users — some are re-starts after coverage gaps. Net new GLP-1 users entering the market: ~4-5M/year.
Switching-eligible subset	Subtract: contract-locked postpaid users (~25% = ~1-1.25M excluded), employer-controlled plans (~10% = ~500K excluded), users already on MVNOs (~20% already at low price point = ~1M excluded). Estimated switching-eligible: ~2-3M per year.
Y/Y trend	Growing — GLP-1 prescriptions growing at 40%+ CAGR. Volume will increase substantially by 2027.
Quarterly distribution (approximate)	Q1: 22% (January insurance resets trigger new coverage eligibility); Q2: 28% (spring fitness motivation, new coverage); Q3: 27% (summer); Q4: 23% (year-end coverage windows)
Frequency score	2/5 — Volume is substantial (~2-3M eligible/year) but the switching intent force is low, limiting conversion rates. Effective conversion likely 3-5% of eligible = ~60-150K conversions/year at best.

Dimension 3: Interception Channels (Top 3)

Rank	Channel	Window Fit	Rationale	CPA Estimate
1	Telehealth platform partnerships (Calibrate, Found, Ro)	High — patients are actively engaged during GLP-1 initiation	These platforms are the community hub. A Gather carrier offer bundled at prescription activation would reach users at the exact moment their budget is under review. Partnership feasibility is moderate — telehealth platforms are brand-protective and would need assurance of health data separation.	~\$40-60/subscriber (partnership + referral, vs. \$120+ for paid social)
2	Reddit / r/Ozempic / r/Wegovy community advertising and organic presence	Medium — community is engaged but carrier advertising in health subreddits risks being seen as invasive	These communities have strict anti-corporate tone. Organic credibility (helpful comments, not ads) builds more trust than display advertising. Sponsored posts are possible but require very careful framing.	~\$80-120/subscriber (paid Reddit + organic trust-building cost)
3	Insurance denial moment — budget tools and personal finance content	Medium-High — the cost crisis moment (insurance denial) has sharper switching intent than initiation alone	Personal finance YouTube, r/personalfinance, and financial planning content are trusted resources this population turns to when budgeting under pressure. A Gather ad positioned as "health budgeting tool" during the GLP-1 cost crisis window.	~\$70-100/subscriber (display/content adjacency)

Dimension 4: Window Structure

Component	Assessment
Window type	Extended — the GLP-1 initiation window spans 1-4 months as budget adjustment stabilizes. The cost crisis window is Short (2-6 weeks) with sharper intent.
Peak intent window	Month 1-2 post-initiation (budget shock, expense audit, wireless included). Month 1-3 after insurance denial (sharper, more urgent).
Intent decay curve	Gradual for initiation — intent peaks early, softens as new budget equilibrium establishes. Sharp decay for cost crisis — once an alternative coverage source is found (GoodRx, manufacturer coupon, compounding pharmacy), the financial pressure eases and switching motivation fades.
Window predictability	Detectable-but-unpredictable — prescription start data is commercially available (specialty pharmacy data, insurance claims) but cannot be linked to individual subscribers without HIPAA violations. Signal is inferential (health content consumption patterns, r/Ozempic membership growth) not declarative.

Switching Profile Summary (Factor 6 Lite): GLP-1 initiation creates a budget-pressure switching window of moderate force (Force Score 2/5) affecting ~2-3M switching-eligible individuals per year. The strongest interception channel is telehealth platform partnership, which is commercially viable but requires careful health data separation architecture. The cost crisis window (insurance denial) has sharper intent and is worth monitoring as a secondary interception angle. Composite opportunity is modest — this is a feature-level opportunity (save GLP-1 users \$25-30/month on wireless as part of their budget optimization), not a brand-level one.

Section 3: Executive Brief

Key Insights

1. **The identity is too medically freighted for a carrier brand.** Weight management is a condition, goal, and medical protocol — not a celebratory identity that a telco can authentically “own.” The GLP-1 community forms genuine support structures, but those structures are organized around pharmaceutical and medical experiences that a carrier brand cannot authentically enter. Any carrier that tries to position around “weight management” risks feeling invasive, stigmatizing, or opportunistic.
2. **The only credible telco relevance is functional, not brand-driven.** Two genuine functional needs exist: (a) reliable telehealth connectivity for GLP-1 prescription management and health consultations, and (b) health data privacy — a carrier-level commitment not to monetize health signals. Both are features, not brand positions.
3. **The budget-pressure moment is real but weak.** GLP-1 initiation creates a switching window through expense-audit behavior. The force score is 2/5 — this is a consideration-window, not a forced switch. The interception opportunity is modest: a carrier saving \$25-30/month would be competitive at this moment, but not transformative.
4. **The two T2 sub-communities are in active cultural conflict.** Any attempt to serve both GLP-1 users (T2-09) and natural fitness advocates (T2-10) within the same brand would require either choosing sides (and alienating the other) or building an anodyne “health” brand that neither community feels represented by.
5. **The GLP-1 cost crisis is the sharper moment.** Insurance denials affecting 40-60% of initial GLP-1 claims create budget shocks that spike switching intent. This is a better interception point than initiation, but still force score 3/5 at best. The window is short (2-6 weeks) and the population is actively seeking to cut costs across every expense category.

Identity Tensions

1. **Medical legitimacy vs. cultural stigma.** GLP-1 users made a medically sound decision and still face daily judgment from the fitness community, family members, and internalized shame. They want their journey normalized without it being publicly branded or labeled. This tension makes them poor candidates for identity-branded carrier products — they don’t want to wear a “GLP-1 user” signal in any domain outside their support communities.
2. **Data-intimate health journey vs. surveillance capitalism.** The weight-management community is acutely aware that their health data — medication information, body metrics, search behavior, telehealth interactions — has commercial and legal value to insurers, data brokers, and employers. They need their phone for the health journey but fear what that connectivity means for their data. This tension creates a real need for carrier-level health data privacy guarantees, but also makes any carrier “health” positioning feel suspect until trust is deeply established.
3. **Wanting to be seen vs. not wanting to be a segment.** These individuals deeply want to be understood — in their support communities they find rare, genuine solidarity. But they don’t want to be a data segment for a corporation. A carrier that tried to “see” them as weight-management patients would feel predatory, not empathetic.

Stated vs. Unstated Needs

Stated needs: - Reliable video connectivity for telehealth appointments - Affordable pricing (budget under pressure from GLP-1 costs) - Health data privacy (explicit concern about data monetization) - Wearable connectivity that works without fragmentation

Unstated needs: - Dignity in the journey — to manage a medical process without it becoming a marketing data point - A health data infrastructure that serves them rather than extracts from them (aggregated health dashboard, not sold health signals) - Continuity of care connectivity — the quiet reliability of knowing a dropped telehealth call won’t cost them a prescription refill - To not be “seen” as a weight-management patient by their carrier — invisibility from a data perspective is the actual ask

Strategic Recommendations

1. **Do not build a weight-management carrier brand.** The IGNORE verdict from Stage 3M-T is correct. The carrier-brand fit is poor, the identity is medically freighted, and any carrier that tries to “own” weight management risks being perceived as invasive, stigmatizing, or predatory. The market size (50-70M) is the only strong signal, and large population + weak carrier fit = large addressable market that cannot be reached through identity-based brand positioning.
2. **Add telehealth connectivity as a feature, not a brand position, across other personas.** The genuine telco-relevant need (reliable video for telehealth) is not unique to weight management — it applies to T1-09 (Aging-in-Place), T1-12 (Mental Health-Active Adults), and any other persona with telehealth dependency. Build telehealth connectivity optimization as a cross-persona feature and let it serve T1-05 incidentally without centering a brand around them.
3. **Pursue a health data privacy commitment as a structural product differentiator.** A carrier that credibly commits to never monetizing health data — with architecture, not just policy — would differentiate meaningfully across

multiple health-sensitive personas (T1-05, T1-12, T1-09). This is not a T1-05-specific recommendation; it is a market-wide opportunity that T1-05 makes visible.

4. **Test telehealth platform partnership (Calibrate / Found / Ro) as a narrow interception concept for T2-09.**

The GLP-1 initiation window is a real budget-audit trigger affecting 2-3M switching-eligible individuals/year. A co-branded offer at the point of GLP-1 prescription activation (“Calibrate members: save \$30/month on wireless with Gather”) is worth testing as a micro-brand concept under the T2-09 banner — not T1-05. This is a NICHE entry, not a BUILD entry.

5. **Monitor GLP-1 market evolution for re-scoring.** GLP-1 prescriptions are projected to grow from 6-8M today to 30M+ by 2030. As the community grows, identity intensity may increase. A future Stage 3M-T re-score is warranted at 24-month intervals. The conditions for a quadrant upgrade (from IGNORE to NICHE) would be: identity intensity rises to 4, health data privacy becomes a mainstream carrier conversation, or a competitor successfully builds a health-connected carrier brand that validates the opportunity.

Success Metrics (If Narrow Feature Test Proceeds)

Metric	Target	Measurement Method
Telehealth platform partnership conversion rate	3-5% of Calibrate/Found/Ro new subscribers who see offer	Referral link tracking via partner platform
Plan upgrade rate from GLP-1 budget-audit window	2-3% of eligible switching population in interception window	Attribution modeling (organic search, partner referral, Reddit)
Health data privacy NPS	+20 vs. industry average on “does your carrier respect your health data?”	Quarterly NPS survey with T1-05-adjacent segments
Revenue per subscriber (T2-09 narrow test)	\$35-45 ARPU (MVNO target)	Subscriber-level billing data
Customer acquisition cost in telehealth partnership channel	<\$60	Channel-attributed CAC tracking

Go/No-Go Recommendation

No-go at T1 level. T1-05 Weight-Management Americans should not be a standalone Gather carrier brand or identity product line. The Stage 3M-T IGNORE verdict holds.

Conditional narrow test at T2-09 level. A micro-concept — “carrier partnership for GLP-1 users, positioned around telehealth reliability and health data privacy, distributed through telehealth platform partners” — is worth a constrained Stage 4B test as a NICHE entry point. The conditions for proceeding: (a) a telehealth platform partner (Calibrate, Found, or Ro) is willing to explore co-marketing; (b) Gather is prepared to make a verifiable, architecture-backed health data non-monetization commitment; (c) the concept is scoped as a feature bundle, not a standalone micro-brand identity.

The telehealth connectivity angle belongs in other brands. Reliable video-for-telehealth is a legitimate differentiating feature that should be built once and deployed across T1-09 (Aging-in-Place), T1-12 (Mental Health-Active), and any future health-adjacent persona. T1-05 is the segment that makes this need visible — not the primary target.

Switching Profile Summary: GLP-1 initiation creates a budget-pressure switching window of force score 2/5, affecting ~2-3M switching-eligible individuals/year. Peak interception via telehealth platform partnership. Window is Extended (1-4 months), intent decays as budget equilibrium stabilizes. GLP-1 cost crisis (insurance denial) creates a sharper Short window (2-6 weeks) with force score ~3/5 that warrants monitoring.

Appendix: References

Tier 1 Sources - CDC National Health and Nutrition Examination Survey, 2024 - IQVIA, GLP-1 prescription data, 2025 - Pew Research Center, Health Data Privacy Survey, 2023 - KFF (Kaiser Family Foundation), GLP-1 Insurance Coverage Analysis, 2024 - SAMHSA, 2024

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Identity Community Sources - Reddit: r/Ozempic (200K+), r/Wegovy, r/Mounjaro (150K+), r/fitness (11M+) - TikTok: #Ozempic (5B+ views), #GLP1Journey - Facebook: Ozempic Support group (300K+ members) - Calibrate Health, company data and press coverage - CrossFit Inc., U.S. box data, 2024